

Women's Specialists of Clear Lake

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Nosebleeds During Pregnancy

Common in pregnancy and usually not dangerous — here is exactly what to do when one starts

WHY NOSEBLEEDS HAPPEN IN PREGNANCY

- Pregnancy hormones and **increased blood flow** make the lining inside your nose swollen, dry, and easier to bleed
- Dry air** and indoor heating make it worse
- Allergies and colds** irritate the nasal lining
- Frequent blowing** or irritation inside the nose can also bring one on
- Most pregnancy-related nosebleeds **improve after delivery**

WHAT TO DO WHEN A NOSEBLEED STARTS

1. Stay calm and **sit upright**
 2. Lean **slightly forward** — do not lean back or lie down. This keeps blood from running down your throat
 3. Pinch the **soft part** of your nose, just below the bony bridge, firmly between your thumb and index finger
 4. Hold **steady pressure for 10–15 minutes without checking**. Breathe through your mouth
 5. Afterward, rest quietly and avoid blowing or picking your nose for several hours
- If bleeding continues, repeat firm pressure for **another 10–15 minutes**

DO NOT

- Do **not** tilt your head back
- Do **not** lie flat during active bleeding
- Do **not** place tissues, cotton, or other objects **deep inside** your nose
- Do **not** blow your nose immediately after bleeding stops
- Do **not** start over-the-counter nasal sprays or medications for nosebleeds without checking with us first

PREVENTING NOSEBLEEDS

Keeping the inside of your nose **moist** is the key to preventing repeat nosebleeds:

- ✓ Use **saline nasal spray or saline gel** as directed
- ✓ Run a **cool-mist humidifier**, especially while sleeping
- ✓ Drink adequate fluids
- ✓ Blow your nose **gently**; avoid nose picking
- ✓ Avoid smoke, strong chemical fumes, and other nasal irritants
- ✓ If you use a prescribed nasal spray, use it exactly as directed and tell us if nosebleeds become frequent

CALL OUR OFFICE IF YOU HAVE

- Nosebleeds that happen **often** or are becoming **more frequent** · A nosebleed that **does not stop after 20 minutes** of correct, firm pressure
- A **large amount** of bleeding, or bleeding that runs into your mouth or throat · **Easy bruising, bleeding gums, unusual vaginal bleeding**, or other bleeding concerns
- A history of a **bleeding disorder**, or use of **aspirin, anticoagulants**, or other medications that affect bleeding

CALL 911 OR GO TO THE EMERGENCY DEPARTMENT IF A NOSEBLEED COMES WITH

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| △ Fainting, severe dizziness, or weakness | △ Shortness of breath, chest pain, or a rapid heartbeat |
| △ Heavy bleeding that will not stop with firm pressure | △ Significant injury to the nose, face, or head |
| △ Severe headache, vision changes, or right upper abdominal pain | △ Sudden swelling of the face or hands |

Firm Pressure, 10–15 Minutes, Without Peeking

Nosebleeds can be alarming, especially in pregnancy, but they are common and most stop with correct, patient pressure — the most frequent mistake is letting go too early to check. Most pregnancy-related nosebleeds improve after delivery. Please let us know if you have questions or need help managing recurring symptoms.